جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : عايده عريان متى ميخائيل
المحافظة : الجيزه **الحي / المدينة :** الصف **العنوان :** الصف
رقم البطاقة : 25712250103187 **رقم التليفون :** 01011445332 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 25-12-1957 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي علاج تدعيمي وتلطيفي [6579] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: عايده عريان متى ميخائيل
National ID: 25712250103187

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Supportive and palliative Treatment

التاريخ : 05-07-2026	**الوقت :** -- --:--
	الطبيب المعالج : د/شيماء لاشين


جمهورية مصر العربية
بطاقة تحقيق الشخصية
عايده
عريان متى ميخائيل
١٣ ش عمربن الخطاب - عزبة اللوره
الخصوص - القليوبية
٢٥٧ ١٢ ٢٥ ٠١ ٠٣١ ٨٧
LN5532117


٢٥٧١٢٢٥٠١٠٣١٨٧
٢٠٢٥/١٢
ربة منزل
أنثى
مسيحية
أرملة
١١
البطاقة سارية حتى ٢٠٣٢/١٢/١٧

A-Patient ID:
1- Name : [redacted] Sex: Female 3- Medical number: 8318/305
Old Med: 533-2024
2-ID number: 25212256103187 Patient Age : 66 Y



Family history [redacted] Phone 01011145332 BMI [redacted] REASON OF NBS [redacted]
marital status [redacted] NRS Score [redacted] GROUP [redacted] Date of NBS 01/03/2024
Co morbidity non-diabetic Menopausal status [redacted] Initial STAGE [redacted]

Breast Imaging :
Rt breast mass with ipsilateral axillary lymphadenopathy BIRADS 5

Mets Workup :
bilateral pulmonary nodules likely metastatic
Rt breast subareolar ildefined parenchymal soft tissue lesion with ipsilateral axillary lymphadenopathy

BX	ER	PR	Her 2 Neu	SISH	KI67
IDC	3/8	8/8	2	Not Amplified	25

T	N	M
C14B LOCALIZED SKIN	1	1 PULMONARY

Sonomammography
LEFT BREAST LIQ MASS 3.8CM ,INTRADUCTAL EXTENSIONTO NIPPLE , OVERLYING SKIN THICKENING, NIPPLE RETRACTION, PATH AXILLA

PATIENT HISTORY
BONE SCAN

TREATMENT PLAN
1ST LINE METASTATIC AROMATASE INHIBITOR

MEDICAL ONCOLOGY	[redacted]	RADIOLOGY	[redacted]
SURGERY	[redacted]	PATHOLOGY	[redacted]
Psychiatric	[redacted]	Nutrition	[redacted]

Name: عايده عريان متري	
MR. : 2417	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient presents with metastatic breast cancer to the lung, with a history of HER2-negative disease diagnosed in March 2024. The multidisciplinary team (MDT) in April 2026 recommended an aromatase inhibitor (AI) regimen, and a bone scan was performed with no evidence of skeletal metastases. The patient was started on Prolia (denosumab) and Femara (letrozole) with supportive care for three months, as noted in the April 2026 plan.

In July 2026, the patient's disease progressed despite the three-month course of Femara and supportive therapy, as indicated by a vague description of progression on the bone scan. The patient was taken off the current endocrine therapy, and a free CT chest was ordered. The MDT will review the CT chest results and consider further systemic therapy options if progression is confirmed.

تحريرا في ,

التاريخ :	19-09-2026	**الوقت :**	

الطبيب المعالج : نهي سعيد علي محمد

يعتمد ,

tient Name	: الأستاذة / عايده حربي متى موخائيل	Request No.	: 13002418
tient No.	: 25712250103187	Request Date	: 27/01/2024
h No.	: 378/24	Verification Date	: 13/02/2024
x	: Female	Financial Category	: اورام دار السلام - مجانيه
e	: 66 Y	Department	: Out Patient
ferreing Doc.	:	IHC No.	:
		Registered by	: .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

ical Data: Left breast mass.
ure of Specimen: Tru-cut biopsy.

hnique:
ections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and natoxylin as counter stain. The sections treated against:
R clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9).

sults:
T (Positive control showed appropriate reactivity):
R: Strong nuclear reaction in 90% of tumor cells (Score 8/8).
R: Strong nuclear reaction in 70% of tumor cells (Score 8/8).
er2: Moderate complete membrane reaction in >10% of tumor cells (score 2+).
67: 25% nuclear reaction of tumor cells.

nclusion:
t breast mass, Tru-cut biopsy:
asive duct carcinoma, Grade II.
:
R: Positive (score 8/8).
R: Positive (score 8/8).
er2: Equivocal (score 2+) pending SISH.
i67: Positive in 25% of tumor cells.
or IPT (SISH).

oring system: "ER, PR"

Verified by :
DR.Menna Nabil

غرفة رقم (1) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (1) - First Floor-Building No. 987- Nile Coroniche - Misr El Qadema - Cairo, Egypt

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

Patient Name: Aida Erian Matta Mikhail	Date : 15/01/2024
Exam : MAMMOGRAPHY BOTH	Age : 66 Year(s)
Medical ID. 25712250103187	Pat. ID: 46077

DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

- left breast palpable mass

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Left Breast:
  - Scattered glandular tissue (ACR b)
  - Left breast LIQ spiculated high density mass lesion harbouring miero-calcific foci, measures 3.7x3.2 cm.
  - U/S revealed correlate malignant-looking mass lesion at 6-7 o'clock, measuring about 3.8x3.2 cm, associated with intraductal extension reaching the nipple as well as overlying skin thickening and skin/nipple retraction.
  - Pathological enlarged level I left axillary LN with focal cortical thickening , measures 2 x 1.3 cm.
  - No pathological enlarged infra or supraclavicular lymph nodes.
- Right Breast:
  - Scattered glandular tissue (ACR b)
  - No suspicious micro calcifications or architectural distortion detected.
  - Scattered benign calcific foci.
  - Complementary ultrasound no cystic or solid mass lesions.
  - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- ***Left breast LIQ malignant-looking mass with ipsilateral axillary lymphadenopathy (BIRADS 5), for histopathological examination .***
- ***Right breast: Adenosis and benign calcifications (BIRADS 2)***

Signature Dr. Peter Nashat

15/01/2024 12:55 PM

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
شعبة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
قسم الأشعة

Patient Name: Aida Erian Matta Mikhail	Date : 15/02/2024
Exam : C.T CHEST	Age : 66 Year(s)
Medical ID: 533/2024	46077

NON-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

** Reported case of left breast cancer.

Findings:

I Chest:

- Left breast subareolar ill-defined parenchymal sft tissue lesion.
- Left axillary level I and II enlarged lymph nodes, largest at level I measures about 1.6 x 1.1 cm in axial dimensions.
- Bilateral pulmonary nodules seen at bilateral upper and lower lung lobes, the largest is seen at right lower lobe posterior basal segment measures about 1.1 x 0.9 cm in axial dimensions.
- No areas of ground glass attenuation, consolidation or cavitary lesions.
- No CT evidence of pathological mediastimnal lymph node enlargement.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross abnormalities.

II Abdomen & pelvis:

- The liver is enlarged showing homogenous parenchymal attenaution. No definite focal lesions on non-conrast basis.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal non contrast CT appearance of the spleen, pancreas, kidneys and suprarenals glands.
- Mid left ureteric radiodense stone is seen measuring about 4 mm in its CC diameter with subsequent mild left pelvicalyceal system dilatation and small sized left kidney and cortical thining out. Small left lower calyceal stones measures about 5 mm with mean density about 200 HU.

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
الهيئة العامة للخدمات الطبية المتخصصة
مستشفى دار السلام للأورام
إدارة الأشعة

Patient Name: Aida Erian Matta Mikhail.	Date : 15/02/2024
Exam : C.T CHEST	Age : 66 Year(s)
Medical ID: 533/2024	46077

- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.

III THE EXAMINED SKELEOTN:

- No sizable lytic or scelerotic leisons.

Opinion:

- **Bilateral pulmonary nodules, likley metastatic.**
- **Left breast subareolar ill-defined parenchymal sft tissue lesion with left axillary lymphadenopathy, likley metastatic.**
- **No hepatic or osseous deposits.**
- **Hepatomegaly.**
- **Left mid ureteric stone with subsequent atrophic chnages of the left kidney and left renal stone.**

Dr. Ali Elsisy, MSc
Dr. Reham Anwar, FEBR, MD

Patient Name	السيدة/ عايده عريان متى	Patient ID.	1026022010079
Age / Sex	68 Years / Female	Request Date	20-Feb-2026 03:09 PM
Referred By	Herself	Printed In	20-Feb-2026 06:35 PM

BIOCHEMISTRY REPORT

Kidney Functions :

				Reference Range :
▲ Serum Creatinine	:	1.4	mg/dL	High Normal: 0.6 - 1.1 mg/dL
▲ Serum Uric Acid	:	6.1	mg/dL	High Normal : 2.5 - 6.0 mg/dL
▲ Blood Urea	:	45	mg/dL	Female 15 - 45 mg/dL

Doctor's signature:

عبد ال

Thanks

Patient Name	: عايده عريان متى	**Patient Id**	: 10752
Patient Info.	: Female 69Y	**VisitId**	: 372912
Department	: Out Patient	**Collection Date**	: 06/09/2026 14:16:17
Profile NO.	: 533-2024 / 2417	**Reporting Date**	: 06/09/2026 14:45:22

Clinical Chemistry Report

Test		Result	Unit	Reference Range
Kidney Function				
Serum Creatinine	:	H 1.29	mg/dL	0.5 - 0.9
Calcium Total	:	9.79	mg/dL	8.8 - 10.6

Signature

Dr. Reaam Abdelmonem

العنوان : 987 كورنيش النيل مصر القديمة ــ رقم خدمة : 0221277647 / 15580

جوستاف روسي

Patient's Name: Aida Aryan Matta
Date: 2026-07-26
Referred by: Prof.Dr. no doctor
Hospital: **Goustave Roussey International Egypt**

MULTISLICE NON CONTRAST CT SCAN OF THE CHEST:

TECHNIQUE: Multislice axial CT images of the chest with multiplanar reconstruction. No IV contrast was used.

FINDINGS:

- Reported case of left breast cancer showing:
- Few left axillary subcentimetric lymph nodes in level I and II
- Bilateral few tiny pulmonary nodules, largest one seen at right upper lobe apical segment measuring about 3mm
- Bilateral basal parenchymal bands.
- no consolidation or cavitation.
- No pathologically enlarged hilar or mediastinal lymph nodes.
- No plural or pericardial collection.
- No gross cardiomegaly
- Bilateral elastofibroma dorsi.
- Dorsal spondylosis.
- Upper abdominal cuts show small hiatus hernia and atrophic left kidney.

Opinion:-

- Few left axillary subcentimetric lymph nodes for U/S correlation
- Bilateral few tiny pulmonary nodules, for follow up.

Dr.Nourhan Ewis

Name : aida eryan mata	Date : 01-01-2026 11:27
Branch : Portable	Medical No. : GRI-46077
Ref.Physician Prof :	Ins.Company :

MULTISLICE C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

I- CHEST:

- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels
- Unremarkable both breasts.

II- ABDOMEN AND PELVIS:

- The liver shows homogenous parenchymal attenuation with a **subcapsular calcific focus measuring 5 mm**. No dilated biliary radicles.
- Right kidney is of normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- **Left kidney is atrophic with ballooned pelvis and a lower renal calyceal stone measuring 4 mm of density 241 HU**
- Normal C.T appearance of spleen, pancreas, suprarenal glands.
- The urinary bladder shows no evidence of gross masses, diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
- No suspicious abdominal or pelvic lymphadenopathy.
- No ascites.

III. SURVEYED SKELETON:

- **Osteodegenerative changes** are noted with no definite osseous lesions

Conclusion:

- **Atrophic left kidney with a left renal lower calyceal stone.**
- **Hepatic subcapsular calcific focus.**
- **Osteodegenerative changes.**
- **Otherwise, unremarkable study.**
- **Further contrast enhanced CT/PET CT is advised.**

Much Obliged;

Dr. Zeinab Aly, MD

Patient Name	: عايده عريان متى	**Patient Id**	: 10752
Patient Info.	: Female 69Y	**VisitId**	: 372912
Department	: Out Patient	**Collection Date**	: 06/09/2026 14:16:17
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Dr. Reaam Abdelmonem

العنوان : 987 كورنيش النيل مصر القديمة ــ رقم خدمة : 0221277647 / 15580

جوستاف روسي

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Date: 2026-07-26
Referred by: Prof.Dr. no doctor
Hospital: **Goustave Roussey International Egypt**

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Name : aida eryan mata	Date : 01-01-2026 11:27
Branch : Portable	Medical No. : GRI-46077
Ref.Physician Prof :	Ins.Company :

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Much Obliged;

Dr. Zeinab Aly, MD